

Meeting Summary — September 10, 2026

Participants: Isak Gutierrez, Thomas Abshier, Joelle Ater (joined ~9:03 AM) **Duration:** ~3.5 hours (7:18 AM – ~10:16 AM)

Key Decision: Don't Build Own DME — Use Existing Dealers

The group reversed course on becoming their own DME supplier. The math doesn't work yet:
- Own DME is California-only (not nationwide as previously assumed) - 9–12 months of paying rent + wages before first billable claim (\$45–60K sunk) - Facility must be fully operational before even applying (CMS eliminated grace period Jan 2026) - Break-even requires ~30 units/month sustained — current volume is zero - **Decision: sell through existing DME dealers (Rifton's model), apply for own DME later when volume justifies it**

Pricing Clarity

Wholesale to DMEs does NOT cap future billing. Trade prices to dealers are not “usual and customary” charges. Direct-to-consumer cash sales ARE what set the floor. So: - Wholesale freely to DME dealers at any price — no billing consequences - Sell to institutions and schools — no billing consequences - Be deliberate about direct-to-family pricing — that's the one that follows you

Working economics through other DMEs: - MSRP \$12,000 → Medicare pays 75% = \$9,000 → DME takes 30% = \$2,700 → Ideomotion gets \$6,300 → minus \$3,500 COGS = **\$2,800/unit profit** - At 15 units/month: \$42,000 revenue, minus \$16K payroll = ~\$26,000 net margin

Soft Goods — Critical Path Item

Susan (Mom) will sew ONE prototype set for photos/video/demo. Not production. - ~2 weeks for first set (liberal estimate — includes measuring, cutting, learning) - Bulk cutting would reduce per-set time significantly - She's willing but happy to hand off — “don't worry about offending me” - Sewing machine pedal needs professional repair (\$200ish)

Soft goods outsourcing plan: - Joelle searching locally (Idaho Facebook business groups, seamstress network) - Leonard's overseas contact may handle soft goods too — needs to be asked specifically - Joelle's backpack friend never responded — trying other routes - Need: materials list with sizes, photos of current soft goods, paper patterns from Charlie/Susan - Kangaroo leather question: is it scalable/affordable? What's the next-best material? - Color: black thread and belts only (colored thread shows defects)

Competitor Intel — Rifton's Model Confirmed

Rifton sells through 4 named DME dealers (Adaptive Mall, SpinLife, Rehabmart, Medical eShop) alongside internal direct sales. They do NOT own a DME supplier. They publish LMN templates and funding guides and let local dealers handle state-specific insurance machinery.

Key finding: Rifton reimburses at \$3,000–\$5,000. Biodex does NOT reimburse at \$12,900. Ideomotion at \$12,000–\$16,000 trying to reimburse is a combination neither competitor does.

Rifton uses caregiver injury argument inside medical necessity letters: “the device has the potential of saving future medical costs of caregivers injuring their backs during lifting and transfers.”

Steve Blank / Startup Owner's Manual

Thomas has been reading the book. Key tension identified: - The book says: find what sells before investing in infrastructure (lean startup) - What Ideomotion is doing: building infrastructure before proving sales (big company approach) - Thomas acknowledges this is the opposite of the book's advice, but on a much smaller scale (\$250K vs billions) - **Resolution:** hybrid approach — sell through existing DMEs (lean) while slowly building toward own DME (infrastructure)

Logo Approved

Joelle designed a logo in Canva — clean, simple, the O in “ideomotion” has a swirl suggesting motion. White on black version preferred. - Charlie approved via text (“likes it, reminds him of something Ben drew early on”) - Thomas: “very professional, serious, not whimsical” - Isak: “the circle says motion twice — from the word and from feeling it” - Keep On Truckin’ (R. Crumb) design reserved for stickers/accessories, not main branding - Next: vectorize the logo, consider trademarking - Joelle started CRM in Airtable and project hub in Notion

Flyer Patent Approved

Jim Duncan notified Charlie that the “Flyer” (a walker variant) patent has been approved. Another product for the pipeline.

DME Application — Final Position

Thomas considered applying just to hold a place in the queue (in case CMS freezes enrollment again), but ultimately decided against it: - It's California-only, not nationwide - \$10K in fees + \$60K/year in facility/staff costs before any revenue - The moratorium could return (government-created scarcity), but can't count on selling a DME enrollment for \$1M+ — that depends on the scarcity continuing - **Final call: don't apply now. Revisit when selling 30+ units/month sustained.**

Action Items

#	Item	Owner	Priority
1	Call Medi-Cal + other state insurers — what do they actually pay for walkers/lifts?	Isak	CRITICAL — stops the circular debate

#	Item	Owner	Priority
2	Visit Mom + Dad — collect soft goods patterns, photos, materials list, sizes	Isak	Today
3	Fix sewing machine pedal	Isak/Charlie	This week
4	Build soft goods spec file for Joelle (patterns, photos, materials, sizes)	Isak	After #2
5	Search locally for seamstress/soft goods manufacturer	Joelle	This week
6	Ask Leonard if overseas contact handles soft goods	Isak	This week
7	Call local DME suppliers — how does the process work?	Isak	This week
8	Financing company research — call shortlist	Joelle	This week
9	CRM (Airtable) — add DME vs therapist vs doctor tags	Joelle	Started
10	Notion project hub — meeting notes, to-dos per member	Joelle	Started
11	Vectorize logo	Joelle	This week
12	Get 3D manufacturing drawings from Leonard (not just quoting PDFs)	Isak	In progress
13	Check if Charlie has CAD drawings for soft goods	Isak	Today
14	Product liability insurance quotes	Isak	Still blocking
15	Run logo by Dad for final approval	Joelle	Done (approved)

Key Quotes

Thomas: “We’re not even close to having a company right now.”

Thomas: “The entire Steve Blank method is to spend hardly any money at all and get orders. Find out what actually sells. And we are blocked on that because we can’t sell anything.”

Isak: “Think about Mom and Dad as the prototype people, and think about everybody else as Victory Circle and seamstresses everywhere.”

Thomas: “We’re hog-tied. We’re by law required to do nothing until we can do the volume at a level we don’t even know if we can sell at yet.”

Isak: “It’s like the amount of fuel a rocket ship needs to take off versus the amount it needs to sustain.”

Thomas on the DME: “If we actually grow, our capital needs keep going up. We’re looking at some significant capital investment here.”

Thomas’s Summary — September 10, 2026

Prepared separately by Thomas Abshier using Claude. Included here as a companion analysis.

Ideomotion — Meeting Record and Decisions

Wednesday, 10 September 2026 · Thomas, Isak, Joelle Soft goods and manufacturing · DME strategy · pricing and reimbursement · branding · infrastructure

Summary of what changed today

Three things moved:

1. **The DME decision reversed.** Yesterday’s plan was to apply for your own DME while selling through others. Today’s research showed a DME license is state-by-state, not national, and that the facility must be fully operating before the accreditation survey. The decision is now: sell through other DMEs, defer owning one.
2. **The pricing paradox resolved.** Thomas framed it mid-call as being hogtied — too expensive to sell retail, too cheap to bill. The resolution that surfaced at the end: the constraint applies only to direct-to-consumer sales. Wholesale and institutional pricing are free.
3. **The soft goods bottleneck was correctly identified as the gating item for the entire business,** not just for the MVP.

1. Soft goods — the rate-limiting step

Where it stands

Susan estimates two weeks for one complete set. At that rate the existing ten machines take roughly half a year to become sellable. Thomas’s read is correct and the group agreed: finding another producer is not optional.

Two qualifications worth holding onto. The two-week figure is deliberately liberal — it was given as room to start, not as a measured rate. And cutting and measuring is the bulk of it; batching the cuts assembly-line style could bring a set down to a few days. We won't know the real number until the first set is done.

Decisions made

- Susan produces one complete set. Purpose: photography, video, demonstration, and — critically — a physical reference sample. Not production.
- Cut patterns for all ten now, regardless of who sews them, so that on sale a unit can ship within a week.
- Black belts and black thread for this batch. Imperfect stitching disappears against black. Colored belts expose it, and the first photographs are not the place to discover that. Revisit color once a professional shop is producing.
- Sewing machine goes in for service — sticking pedal, a couple hundred dollars, a couple of days. This changes both quality and speed and is worth doing before the set is started.
- Charlie and Susan are the prototype shop, not the production line. Susan has said plainly she'd be glad to hand off sewing and won't be offended.

The reference-sample principle

Thomas's point here is the most useful thing said about manufacturing today, and it's backed by Charlie's own experience of a prior soft-goods vendor delivering the wrong foam thickness and texture against a written spec.

Send the contract manufacturer a finished physical set along with the drawings: "this is what it looks like when you're done." An object lesson is unambiguous in a way a written specification never is. A vendor can satisfy every dimension on a drawing and still produce something unacceptable.

Sequencing note: Susan's one set is simultaneously the photo prototype and the manufacturing reference sample. Those are the same physical object. Photograph and film it before it ships anywhere.

Sourcing — three parallel tracks

Track	Owner	Status
Local seamstresses via the area business-owner Facebook group	Joelle	Fastest near-term option; needs only photos and a materials list to start
Backpack contact	Joelle	Went unresponsive after repeated nudges; deprioritize
Leonard's Africa procurement contact	Isak / Leonard	Was on vacation. He must be told explicitly that soft goods are in scope — this was never communicated and shouldn't be left to assumption

The file problem

The soft goods patterns are not in any shared folder. Leonard does not have them. Charlie may have CAD drawings — Thomas recalls him spending weeks or months on soft-goods drawings — but this is unconfirmed, and paper patterns definitely exist.

Joelle's interim ask is the right unblock: she doesn't need CAD to start shopping. A materials list, basic dimensions, and one good photo showing most of the soft goods is enough to solicit quotes. Isak to assemble that file and visit Charlie and Susan to collect specs and photograph the paper patterns.

Materials question raised

Joelle flagged kangaroo leather — cost and supply chain viability at scale. Needs a materials substitution ladder from Charlie: what's essential to the product, what's preference, and what's the next best thing if the first choice doesn't scale.

2. DME strategy — decision reversed

Yesterday's plan: Apply for your own DME now while selling through third parties during the wait.

What surfaced today

A DME license is state-by-state, not national. This was the fact that turned the decision, and Thomas named it directly: "I was thinking it was nationwide, but it's not. That was simply wrong."

The three layers behave differently:

- **Medicare billing: travels.** DMEPOS claims go to the DME MAC for the jurisdiction where the beneficiary lives, not where the supplier sits. A California-enrolled supplier can bill for a patient in Ohio.
- **State licensure: does not travel.** Supplier Standard 1 requires compliance with all applicable state licensure requirements, and many states require out-of-state DME suppliers to register. California publishes Form CDPH 86790 specifically for out-of-state Home Medical Device Retailer registration. Illinois runs 10–12 weeks plus a \$10,000–\$50,000 bond for out-of-state suppliers.
- **Medicaid: fifty separate programs, fifty enrollments.** A Medi-Cal provider number does nothing in Texas.

Net effect: owning a California DME would let you serve California well and everywhere else badly.

The facility must be real before the application, not after. The gating item isn't CMS — it's accreditation. CMS eliminated the operational grace period effective January 1, 2026; new locations must be fully surveyed before accreditation is granted, with no temporary status. Surveys are unannounced and include records review, staff interviews, beneficiary interaction, and inventory inspection. You cannot be surveyed in an empty room.

The real sequence:

Month	What must be true
0	Lease signed, build-out done, permanent signage up, landline installed and listed, records storage in place, policies and procedures written

Month	What must be true
0–1	Person hired and working posted hours (~30 hrs/week); California HMDR and exemptee license obtained
1–2	Apply to accrediting organization; inventory on hand
3–6	Unannounced survey, then accreditation decision
6	Now you can file CMS-855S
9–12	Enrollment completes, PTAN issued, first billable claim

Nine to twelve months of rent and wages before one dollar comes back — roughly \$45,000–\$60,000 spent before the first claim, on top of about \$10,000 in fees. A failed survey restarts the clock and you keep paying through the corrective action.

Rifton doesn't own a DME. They sell through Adaptive Mall, SpinLife, RehabMart, and Medical E Shop — four named dealers alongside an internal direct sales team — and publish LMN templates and a funding guide so local dealers and families handle the state-specific machinery. That's an existing, working proof that the model doesn't require owning one.

Decision: Sell through third-party DMEs. Defer owning one until there is volume and capital to justify it.

One open idea, and why it doesn't work

Thomas floated filing the California application now simply to hold a place in the queue, in case CMS freezes enrollment again.

There is no queue to hold. The CMS-855S comes at month six, after accreditation, and accreditation requires a surveyed, staffed, operating facility. You cannot file early and wait. Getting a place in line means signing a lease and hiring someone today — which is the whole expense you just decided to defer.

The re-freeze risk is real but it's a reason to watch, not a reason to spend. If CMS announces another moratorium, applications already pending are exempt — but “pending” requires having filed, which requires the facility. The genuinely cheap hedge is a calendar reminder to check the Federal Register quarterly.

3. The pricing rule that resolves the paradox

Thomas stated the bind clearly mid-call: the product is too expensive for anyone to buy at retail, and if it were priced where people would actually buy it — around \$6,000 — that becomes the price you must bill. Hogtied in both directions.

The resolution: the constraint applies only to direct-to-consumer sales.

Sale type	Effect on future billable rate
Direct to a family, cash	Caps your billing near that price, permanently, until published prices change
Wholesale to a DME dealer	None. A trade price is not your usual and customary rate. Every manufacturer has one

Sale type	Effect on future billable rate
Sales to institutions and schools	None

So: wholesale freely; be deliberate about direct-to-consumer pricing, because that's the one that follows you.

4. Reimbursement research — and a combination nobody else runs

How Rifton operates:

- Publishes sample Letters of Medical Necessity for every product (Pacer for school, Pacer for home care, Standard, Wave, Compass Chair, HTS), plus a funding guide aimed at families and therapists
- Bills everything under E1399, miscellaneous DME — no standard fee schedule, individual consideration, LMN required each time
- Sells through named DME dealers alongside an internal direct sales team
- The engine underneath is pediatric: EPSDT requires medically necessary DME for Medicaid beneficiaries under 21 even when not covered in the state plan. In schools, equipment is requested through an IEP and funded by a mix of Medicaid, district budgets, and private insurance

The warning worth taking seriously:

Rifton reimburses, at \$3,000–\$5,000. Biodex doesn't reimburse, at \$12,900. Ideomotion is proposing \$12,000–\$16,000 and reimbursement — a combination neither of them runs.

That's a fork in the road. It may mean the reimbursement market wants a \$5,000 product and the \$12,000–16,000 product belongs in the cash, facility, and VA channels. Or it may mean there's a genuine gap. Either way, Isak's Medi-Cal and state-insurance calls are the highest-value research item on the list — if E1399 individual consideration for a walker lands at a few hundred dollars, the DME channel isn't worth building at all, and the answer changes everything downstream.

A correction to earlier advice

In the 28 August memo the recommendation was to firewall caregiver-injury language out of the federal file entirely. Rifton does the opposite — their Pacer LMN states the device has potential to save future medical costs from caregivers injuring their backs during lifting and transfers, and it appears inside the medical necessity letter.

The refinement: caregiver cost-offset works as a supporting argument alongside a primary patient-benefit justification. What still holds is that it must not be the primary framing, and the words that map to non-covered codes — patient lift, standing frame, transfer device — stay out regardless. Follow Rifton's structure here.

5. Channel sequence — who to approach first

The therapist, not the DME, not the physician.

A DME won't stock an unproven product with no code and no track record — that's inventory risk plus claim risk on their side, and a bad deal for them.

The referral chain: therapist evaluates → physician signs the order → patient goes to an enrolled DME supplier → supplier obtains prior authorization → supplier delivers and bills the payer → payer pays the supplier. The supplier has already paid Ideomotion at wholesale. Their 20–40% buys the claim risk, the denial, the appeal, and a 30–90 day float from delivery to payment — longer with a miscellaneous code.

Joelle's counterpoint is also right and not in conflict: call the DMEs early anyway, to learn the process and the timeline. If onboarding a new product takes months, you don't want to have sold a patient first and left them waiting.

Resolution: therapist-first for demand generation, DME-first for channel intelligence. Both start this week.

6. Economics of the DME channel

Thomas's working model:

- 15 units/month × ~\$3,000 margin = \$45,000/month
- Less current payroll of \$16,000/month = ~\$29,000/month toward inventory and other expenses
- Charlie reportedly sold roughly 15/month at a previous venture, so the volume isn't hypothetical

Working capital is the constraint. DMEs pay 30–90 days from delivery. At 10 units/month and \$3,500 COGS that's \$35,000/month tied up; three months of float is roughly \$105,000. At 15/month it's closer to \$150,000. Growth increases the capital requirement — the faster you sell, the more cash is trapped in units already shipped.

Thomas's estimate of total capital before self-sustainability is closer to \$1M than \$100,000, with a stated preference to avoid outside investors so that margin isn't diverted to servicing an investment.

The item that makes this model contingent: 15 units/month requires 15 soft goods sets per month. Susan cannot produce that and no vendor is identified. The soft goods vendor is now the gating item for the whole plan, not just the MVP. No vendor means no volume; no volume means the DME channel math never gets tested.

7. Branding and logo

Decision: adopt Joelle's concept as the working logo

Wordmark with a swirl in the O conveying motion. Roughly 20 iterations, built in Canva after AI attempts failed.

Reception: Thomas read it as serious, weighty, no-compromise — “it's something you depend on.” Isak liked its simplicity and noted it says motion twice, once from the word and once from the form. Charlie approved, noting it recalled an early drawing of Ben's referencing Muybridge's humans-in-motion series.

Preferred treatment: white on black.

Keep on Trucking (Crumb) artwork — secondary use only

Permitted on equipment, business cards, and effectively everything except t-shirts. Joelle's reasoning for keeping it off the primary mark is right: the people forming a first impression are therapists, physicians, and suppliers, not the geriatric end user who'd recognize the reference. Agreed use: sticker sheets letting customers personalize their unit, plus secondary applications.

Next steps: vectorize, consider trademark filing.

8. Infrastructure and systems

Item	Owner	Status
Airtable CRM	Joelle	Built. Tagged by contact type (DME, therapist, etc.)
Notion workspace	Joelle	Started. Single hub page with subpages for meeting notes, per-member task lists, and links
Liability insurance	Isak	Preliminary questionnaire in progress
Bank account	Isak	Partner agreement in hand; remaining documents to submit
Financing partner	Joelle	List assembled; calls pending on bank account
Website	Isak / Steve	WordPress with Divi. Steve has cPanel access
3D manufacturing files	Leonard	In progress, starting with Traveler
File storage	Isak	GitHub handles CAD poorly. Google Drive vs OneDrive decision pending

9. Open questions

1. What will Medi-Cal and other payers actually reimburse? Everything downstream depends on this. If E1399 individual consideration lands at a few hundred dollars for a walker, the DME channel isn't worth building.
2. Can a \$12,000–16,000 product also be a reimbursement product? Rifton reimburses at \$3,000–5,000; Biodex doesn't reimburse at \$12,900. This combination is unproven.
3. Where are the soft goods patterns, and are they in CAD?
4. 250 lb capacity — conservative or structural? Third meeting running.
5. Who sews at volume? No vendor identified, and the whole 15-units/month model depends on one existing.